

HOW TO OPEN A HOME CARE AGENCY in North Carolina

A DHSR-licensed Home Care Agency (in-home aide services) — private pay & Medicaid.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The North Carolina model — read this first

In North Carolina, an agency that sends aides into people's homes to help with daily living — bathing, dressing, meals, light housekeeping, companionship — is licensed by the NC Division of Health Service Regulation (DHSR) as a Home Care Agency under NC General Statutes Chapter 131E, Article 6, Part 3 and rules at 10A NCAC 13J. If you provide hands-on personal care (in-home aide services), you need the full Home Care Agency license. As of a 2023 law, a business that does ONLY companion, sitter, and homemaker work with no hands-on care can instead use a lighter “Home Assistance Services Provider” registration. This guide covers the full Home Care Agency license (the path that lets you do personal care and bill Medicaid), and flags the lighter track. It's broken down so a first-timer can follow it.

Two tracks — pick by whether you touch the client. *Full Home Care Agency license (this guide): required to provide hands-on personal care (in-home aide services) — and the only path that lets you bill Medicaid Personal Care Services. Home Assistance Services Provider registration (2023 law, G.S. 131E-138(h)): a lighter, non-licensed track for companion/sitter/homemaker-ONLY businesses that provide no hands-on care — they carry liability insurance, tell clients in writing they don't provide hands-on care, run background checks, and attest annually, but can't bill insurance. If you want to do personal care or take Medicaid, you need the full license.*

Accreditation — optional, and ACHC is in your backyard. *North Carolina does NOT require accreditation for a home care license. But accreditation (ACHC — which is headquartered in Cary, NC — CHAP, or The Joint Commission) is often preferred by VA and managed-care contracts and helps you win referral partners. Every PolicyReady product is built to those standards behind the scenes; treat accreditation as a goal for year two (Part 9).*

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written for someone who has never opened an agency — where we name a thing, we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with DHSR, NC Medicaid, and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

1	Decide this is your business	A licensed in-home aide agency — recurring, hourly revenue (Part 1)
2	Form the business & insure	Entity, EIN, NPI, and the right insurance & bonding (Part 3)
3	Get the DHSR license	Pre-application training, the application, the \$560 fee, the survey (Part 4)
4	Set up your agency director	The director qualifications + your RN for competency (Part 5)
5	Recruit & screen aides	Competency, NC DPS & Health Care Personnel Registry checks, TB (Part 5)

6	Set up EVV & policies	Sandata EVV, P&P Manual, your survey readiness (Part 6)
7	Serve private-pay clients	Start earning while Medicaid enrollment is pending (Part 8)
8	Enroll for Medicaid	NCTracks enrollment + Personal Care Services (Policy 3L) (Part 8)
9	Get your first clients	Discharge planners, Area Agencies on Aging, CAP/DA (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, all in one place (Part 11)

Rule of thumb. Before you can apply you must show prior home-care ownership OR complete home-care provider training — start that early, along with your entity and RN arrangement. Launch on private pay so you're earning while the DHSR survey and Medicaid (NCTracks) enrollment run in the background.

PART 1

What This Business Is

In short: A DHSR-licensed Home Care Agency providing in-home aide services — personal care and homemaker help in clients' homes. Agency-based, paid by private pay and Medicaid.

North Carolina's home care agency license covers a range of in-home services; a non-medical agency focuses on in-home aide services — personal care (hands-on help with bathing, dressing, grooming, toileting, transferring, eating, mobility) and homemaker tasks (meals, light housekeeping, laundry, errands). NC organizes aide work into In-Home Aide Levels I–IV: Level I is homemaker (no hands-on), Level II is personal care (hands-on), and Levels III–IV cover more complex care.

In-home aide vs. skilled home health

	In-home aide (this guide)	Skilled home health
What it is	Personal care & homemaker help	Nursing, therapy, medical social work
Who delivers it	Trained in-home aides (Levels I–IV)	RNs, LPNs, PT/OT/ST
Certificate of Need	NOT required	CON required for Medicare-certified home health
Your license	Home Care Agency (in-home aide)	Home Care Agency + Medicare certification/CON

This guide is for the in-home aide (personal care/homemaker) agency. Adding Medicare-certified skilled home health later triggers a Certificate of Need — a separate, harder process.

Good news on Certificate of Need. NC requires a Certificate of Need for Medicare-certified home health and hospice — but NOT for a non-medical in-home aide agency. And the moratorium that once blocked new in-home aide licenses was lifted July 1, 2019, so the door is open.

Key terms. DHSR = Division of Health Service Regulation (your licensor). In-home aide = personal care/homemaker worker (Levels I–IV). HCPR = Health Care Personnel Registry (the abuse/neglect registry you must check). PCS = Medicaid Personal Care Services (Policy 3L). NCTracks = NC Medicaid's provider enrollment/claims system. CAP/DA = Community Alternatives Program for Disabled Adults (an HCBS waiver). EVV = Electronic Visit Verification (Sandata). NC LIFTSS = the Medicaid assessment/independent-review contractor. NPI = National Provider Identifier.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. North Carolina is one of the fastest-aging states in the country — its 65-and-over population is projected to keep climbing sharply through the 2030s, concentrated in fast-growing metros like Charlotte, Raleigh-Durham, and the coast, so the pool of families needing in-home help expands every year. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) undercapitalization — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) weak caregiver recruiting and retention — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) sloppy compliance — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Who Regulates You (and What Each One Does)

In short: *DHSR licenses and surveys you, NC Medicaid/NCTracks + the health plans pay you, NC LIFTSS assesses clients, and the DPS and Health Care Personnel Registry screen your staff.*

DHSR — Division of Health Service Regulation (your licensor)

DHSR's Acute and Home Care Licensure and Certification Section issues your home care agency license, writes the 10A NCAC 13J rules, reviews your policies and personnel, and conducts the initial and ongoing surveys. Phone 919-855-4620; DHSR.homecare@dhhs.nc.gov; info.ncdhhs.gov/dhsr/ahc/homecare.html.

NC Medicaid, NCTracks & the health plans

NC Medicaid pays for Personal Care Services (PCS). You enroll as a provider through NCTracks and bill there or through the member's health plan. NC has moved to Medicaid Managed Care Standard Plans (AmeriHealth Caritas, Carolina Complete Health, Healthy Blue, UnitedHealthcare, WellCare); assessments and independent review run through NC LIFTSS (Acentra/KEPRO). NCTracks: 800-688-6694; NC LIFTSS: 833-522-5429.

DPS & the Health Care Personnel Registry (screening)

Before an aide sees clients, you submit a state and national criminal background check to the NC Department of Public Safety, and you check the Health Care Personnel Registry (HCPR) for any finding of abuse, neglect, or misappropriation. Both are required by rule.

EVV — Electronic Visit Verification (Sandata)

For Medicaid personal-care visits, NC requires Electronic Visit Verification. NC's selected vendor for PCS and CAP is Sandata (free for NC Medicaid Direct); managed-care home health uses HHAeXchange or CareBridge depending on the plan. A visit must be verified for the claim to pay. Private-pay work isn't subject to EVV.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. You form it with the North Carolina Secretary of State — filing Articles of Organization (Form L-01) for an LLC costs \$125, and NC LLCs then file an annual report (about \$200 on paper, roughly \$203 online) every year to stay in good standing. File online at sosnc.gov (Business Registration, 919-814-5400). You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

1. **Go to the NC Secretary of State.** File online at sosnc.gov (Business Registration line 919-814-5400).
2. **File Articles of Organization (Form L-01).** The filing fee is \$125. Name the LLC and list a registered agent (this can be you).
3. **Adopt an Operating Agreement.** Set out ownership, management, and how money moves.
4. **Calendar your annual report.** NC LLCs file an annual report — \$200 (paper) or about \$203 (online) — each year.

No newspaper publication in North Carolina. *Unlike some states, NC does not make you publish a notice in a newspaper — forming the LLC is a straightforward online filing.*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

1. **Go to [IRS.gov](https://irs.gov) and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
2. **Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
3. **Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
4. **Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. In North Carolina you need it to enroll with NC Medicaid and bill Personal Care Services (PCS) through NCTracks (Part 8). Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

1. **Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
2. **Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.
3. **Enter your legal name, EIN, and address, and pick your taxonomy.** Choose the taxonomy that fits a home-care / personal-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: Use a bank with lots of NC branches — Truist (headquartered in Charlotte), Bank of America (also Charlotte), Wells Fargo, First Citizens, or a strong local credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

Home care carries real, specific risk — an aide gets hurt lifting a client, something goes missing from a home, or someone alleges mistreatment. General liability alone will not cover all of it, so you need a stack of coverages:

Coverage	What it protects against	Must-have?
General & Professional Liability	Injury/property damage + claims about the care	Yes — required for licensure
Abuse & Molestation	Allegations of abuse by an aide (GL excludes it)	Yes — high exposure
Employee Dishonesty / Surety Bond	Theft from a client's home	Yes — clients & payers expect it
Non-Owned & Hired Auto	Aides driving own cars for/with clients	Yes — critical for home care
Workers' Compensation	Aide injuries (lifting/transferring)	Required in NC at 3+ employees

Real places to get it

Buy through an independent agent or broker who writes home-care programs — carriers active here include Philadelphia (PHLY), CNA, The Hartford, and Markel, plus program managers like Caitlin Morgan and NIP/HomeCareUSA. Find a North Carolina agent at [trustedchoice.com](https://www.trustedchoice.com).

What to ask the insurance agent

"Do you write in-home aide / personal-care agencies — which carriers, and what's the minimum premium?"

"Is abuse & molestation included, plus non-owned auto and a dishonesty bond?"

"What limits do NC Medicaid plans, the VA, or LTC insurers require?"

"When do I need NC workers' comp given my aide headcount?"

PART 4

Get Your DHSR License — Step by Step

In short: Complete the pre-application requirement, submit the application with your policies, pay the \$560 fee, pass the initial survey, and receive your Home Care Agency license.

4.1 Before you apply (the pre-application requirement)

NC requires, before you apply, EITHER proof you previously owned/operated a home care agency OR proof you completed home-care provider training. Review DHSR's Initial Home Care Survey Checklist so you build your policies to what the surveyor will check.

- ☐ Entity formed; EIN and NPI obtained; insurance & bond bound
- ☐ Home-care provider training completed (or prior-ownership proof)
- ☐ A qualified agency director identified and an RN arranged for competency verification (Part 5)
- ☐ Your policies & procedures written (your P&P Manual) and personnel process ready

4.2 The application & survey

- 1. Submit the DHSR Home Care application.** From info.ncdhhs.gov/dhsr/ahc — with your ownership, services, policies, and director information.
- 2. Pay the fee.** The initial application fee is \$560 (renewal \$510; change of ownership \$510) — nonrefundable.
- 3. Complete the application within 12 months.** You have one year from the initial application/fee date to finish.
- 4. Pass the initial survey.** DHSR reviews your policies, personnel files, and (as applicable) client records; after a clean review, the approval letter and license are typically mailed within 5–10 business days.

4.3 Timeline

Between the pre-application training, assembling policies, and scheduling the survey, plan for a few months. Start your Medicaid (NCTracks) enrollment and aide hiring in parallel, and launch on private pay as soon as you're licensed.

PART 5

The Agency Director, RN & Aides — by Position

In short: NC is specific about your agency director, competency verification, and background screening. Here it is, one role at a time.

5.1 Your agency director (10A NCAC 13J .0902)

Every home care agency needs a director. The director must hold a bachelor's degree in health, business, or public administration/science and have at least one year of supervisory or management experience in home care or another licensed health care program. Have a written job description and the director's resume ready for the survey. (This may be you if you meet the criteria, or a qualified hire.)

5.2 RN competency verification

NC doesn't require a non-medical agency to have a full-time Director of Nursing, but a registered nurse must verify each aide's hands-on personal-care competency (mobility/transfers, bathing, toileting, dressing, feeding, continence) per 10A NCAC 13J. If you bill Medicaid Personal Care Services, Policy 3L adds RN supervisory oversight and periodic supervisory visits. Arrange an RN (employed or contracted) from the start.

5.3 Your in-home aides — training & competency

- **Competency-based:** aides must pass a documented competency evaluation for the tasks they perform; NC uses the In-Home Aide Level I–IV framework (Level I homemaker, Level II personal care).
- **Registry note:** for Medicaid PCS and NC's home-care specialty training program, aides generally must meet NC's aide qualification standards (and, for the specialty track, be listed on the NC Nurse Aide I Registry) — confirm your payer's exact requirement.
- **TB testing:** personnel records must include proof of a TB skin test (or chest X-ray) for in-home caregivers.

5.4 Background checks — the system you must run

1. **Submit the NC DPS criminal background check.** Within five business days of a conditional job offer, submit a state and national criminal history check to the NC Department of Public Safety (G.S. 131E-265).
2. **Check the Health Care Personnel Registry.** Confirm the aide has no finding of abuse, neglect, or misappropriation on the HCPR.
3. **Screen against the National Sex Offender Public Website** and honor NC's disqualifying-conviction rules.
4. **Document everything.** Keep dated results in each personnel file — a surveyor will ask.

5.5 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

EVV, Policies & Survey Readiness

In short: Write your policies, set up Sandata EVV for Medicaid visits, and keep your files survey-ready. EVV is the gate to Medicaid payment.

6.1 Your policies & procedures

DHSR expects written policies covering client rights, admission and assessment, the plan of care and supervision, aide competency and training, background checks and the HCPR reporting policy, complaint handling, incident and abuse reporting, emergency preparedness, and infection control. Build them to the Initial Home Care Survey Checklist — it's exactly what the surveyor checks.

6.2 Set up EVV (before your first Medicaid visit)

1. **Use Sandata for PCS/CAP.** NC's free EVV vendor for Personal Care Services and CAP is Sandata; managed-care home health may use HHAeXchange or CareBridge — confirm per plan.
2. **Onboard and train.** Set up your aides, clients, and authorizations.
3. **Capture every visit and bill against it.** A visit must be verified for the claim to pay; fix exceptions promptly.

Private pay is EVV-free. EVV applies to Medicaid visits. Launching on private pay means no EVV until your first Medicaid client — one more reason private pay is the easier on-ramp.

6.3 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Client's Journey — Intake to Discharge

In short: A client is referred (privately or via NC LIFTSS/a health plan), you assess and set up the plan of care, an aide delivers the hours (with EVV for Medicaid), the RN verifies competency and supervises, and you review and adjust.

Stage 1 — Referral & inquiry

A referral comes from a family, a hospital discharge planner, a home-health/hospice partner, or a CAP/DA case manager or health-plan care manager (Part 10). Someone asks whether you can staff care for a specific person.

What you do:

- Take the basics (who, what tasks, hours, where, when, who pays). Confirm the need fits in-home aide services and your capacity.

Stage 2 — Assessment & the plan of care

Do an in-home assessment and build the plan of care: the tasks, schedule, and hours. For Medicaid PCS, the NC LIFTSS assessment sets the authorized hours (adults up to 80/month, plus limited safeguard hours); your plan aligns to it.

Stage 3 — Agreement, competency & start of care

The client (or representative) signs a service agreement (services, schedule, rate/payer, rights, termination). The RN verifies the assigned aide's competency for the client's tasks, and you begin care with a screened, competent aide.

Stage 4 — Delivering the hours (with EVV for Medicaid)

The aide delivers care; Medicaid visits are captured in EVV. Keep visit documentation and protect the client-aide match — consistency drives satisfaction and retention.

Stage 5 — Supervision & quality

Follow your supervision policy (and Policy 3L's RN supervisory visits for Medicaid clients); monitor performance and changing needs; run your quality program; and report any suspected abuse, neglect, or exploitation, plus required HCPR reports.

Stage 6 — Reassessment & changes

As needs change, update the plan of care; for Medicaid clients, coordinate reassessments and new authorizations through NC LIFTSS / the health plan. Keep authorizations current so delivered hours get paid.

Stage 7 — Discharge & transition

When care ends, follow your discharge policy, give proper notice, coordinate with the family and any payer, and document the transition. A good discharge turns families into referral sources.

PART 8

How You Get Paid — Private Pay, Medicaid (PCS), VA & LTC Insurance

In short: Start on private pay and long-term-care insurance the day you're licensed; add Medicaid Personal Care Services once you enroll. NC PCS is billed per 15-minute unit.

Your four revenue streams:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Start immediately; ~\$30/hr in NC (2024); no EVV/enrollment
Medicaid — PCS	Personal Care Services (Policy 3L) + CAP/DA waiver	Up to 80 hrs/month for adults; requires enrollment & EVV
Long-term-care insurance	The client's LTC policy reimburses in-home care	Bill the family or insurer once benefit triggers are met
Veterans (VA)	VA Homemaker/HHA, Veteran-Directed Care, Aid & Attendance	Serve veterans via the Community Care Network (Region 1, Optum)

8.1 Private pay — your launch market

The day your license issues, serve private-pay clients — families paying out of pocket, typically billed by the hour with a visit minimum. North Carolina private-pay non-medical care runs roughly \$30/hour (2024 cost-of-care data; verify the current figure). Set your rate above your fully-loaded aide cost. Long-term-care insurance is part of this stream: many policies reimburse in-home personal care once ADL/cognitive triggers are met (some require a licensed agency) — you bill the family or the insurer.

8.2 Medicaid — Personal Care Services & CAP/DA

NC Medicaid Personal Care Services (Clinical Coverage Policy 3L) pays for hands-on personal care for eligible members — adults may be authorized up to 80 hours a month, with limited additional “safeguard” hours. The Community Alternatives Program for Disabled Adults (CAP/DA) is an HCBS waiver covering personal assistance and in-home aide services for members at nursing-facility level of care. To bill either, you enroll and become a “willing and qualified” provider.

How to get set up to bill Medicaid

1. **Hold your DHSR license first (Part 4).**
2. **Enroll through NCTracks.** Register your NPI and agency; NCTracks 800-688-6694.
3. **Meet Policy 3L requirements.** Aide qualifications/competency and RN supervisory oversight; for CAP/DA, coordinate through the e-CAP system and NC LIFTSS (833-522-5429).
4. **Set up Sandata EVV (Part 6)** before you deliver a Medicaid visit.

8.3 The rate & billing

NC Medicaid pays PCS per 15-minute unit under CPT 99509. The base rate was \$5.96 per 15-minute unit (effective July 1, 2023); an across-the-board 8% PCS rate reduction took effect October 1, 2025, so the current per-unit rate is lower — pull the exact figure from the NC Medicaid Fee Schedule portal before you rely on it. (Congregate-setting PCS moved to a daily per-diem in April 2025.) Bill through NCTracks or the member's plan against verified EVV visits, and confirm the current timely-filing window.

8.4 Veterans

- **VA Homemaker / Home Health Aide (H/HHA):** the VA's in-home program — a VA nurse assesses, then a contracted agency provides the aide. Contract through the VA Community Care Network — North Carolina is in Region 1, administered by Optum (Community Care line 844-839-6120).
- **Veteran-Directed Care:** the veteran gets a flexible budget to hire their own caregivers — you can be their agency.

- **Aid & Attendance:** an increased VA pension (2025–2026: up to about \$2,424/month for a single veteran, \$1,558 for a surviving spouse — verify on VA.gov) applied toward your private-pay care.

8.5 Contracting with the Medicaid managed-care plans

NC Medicaid now runs mostly through managed care, so enrolling in NCTracks is only half the job — you generally also have to contract with the health plans that actually pay for your members. There are two plan families: Standard Plans (the general population) and Tailored Plans (members with significant behavioral-health, I/DD, or traumatic-brain-injury needs, launched July 1, 2024). Where a member is enrolled decides who authorizes and pays for their PCS hours.

1. **Finish NCTracks enrollment first.** Your DHSR license, NPI, and Policy 3L readiness must be in place before a plan will contract with you.
2. **Contract with the Standard Plans in your region.** AmeriHealth Caritas, Carolina Complete Health, Healthy Blue, UnitedHealthcare, and WellCare — each has its own provider-enrollment and credentialing process; join the ones your target clients use.
3. **Add Tailored Plan networks if you serve those members.** The regional Tailored Plans manage behavioral-health, I/DD, and TBI populations — contract separately if you'll staff their members.
4. **Confirm how PCS is administered for each member.** Some PCS stays with NC Medicaid Direct; most runs through the member's plan — verify per member so authorizations and claims go to the right place.

***Why this matters for cash flow.** A signed plan contract is what turns an authorization into a paid claim. Start the contracting conversations early — credentialing with several plans can take weeks, and an out-of-network agency can't bill for a plan's members.*

PART 9

What It Costs & How Long It Takes

In short: Budget the license, the provider training, an RN for competency, insurance & bonding, EVV, payroll float, and (optionally) accreditation. Launch on private pay, then add Medicaid.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
DHSR license	Initial home care agency license	\$560 (renewal \$510)
Provider training	Home-care provider training (pre-application)	Course cost
RN (competency/supervision)	Employed or contracted RN for competency & PCS supervision	Ongoing
Business setup	SOS \$125 + ~\$203/yr report, EIN (free), NPI (free), legal	~\$330 + legal
Insurance & bond	Liability + abuse + non-owned auto + dishonesty bond + workers' comp	Get quotes — annual
EVV & software	Sandata (free) + scheduling/billing/payroll platform	Monthly SaaS
Payroll float	You pay aides before Medicaid/insurers pay you	Working capital
Accreditation (optional)	ACHC (Cary, NC) / CHAP / Joint Commission	~\$7,000–\$14,500 / 3-yr cycle

The cash-flow trap to plan for. You pay aides weekly, but Medicaid and insurers pay weeks later. Private-pay clients smooth this out — line up a business line of credit before you scale Medicaid hours.

9.2 Timeline

Early	Training, entity, RN	Provider training, form the LLC, line up your RN
Weeks	Insurance & policies	Bind coverage & bond; write your P&P to the survey checklist
Submit	DHSR application	Application + \$560; pass the initial survey
Right away	Start private pay	Serve private-pay & LTC-insurance clients
Parallel	Enroll for Medicaid	NCTracks + Policy 3L + Sandata EVV
Ongoing	Referrals & census	Discharge planners, AAAs, CAP/DA case managers

9.3 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$30/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

9.4 The hidden costs new owners miss

- **Payroll before you're paid:** you pay aides weekly, but Medicaid and insurers reimburse weeks later — the single biggest cash drain, and why a line of credit matters.
- **Unbillable time:** recruiting, interviewing, orientation, competency check-offs, RN supervisory visits, scheduling, and travel between clients are all real costs you can't bill to a client.
- **Turnover:** every aide who quits costs you re-recruiting, re-screening (background checks, TB), and retraining — and sometimes the client they served.
- **Compliance overhead:** EVV fixes, HCPR and background-check re-runs, policy updates, and survey preparation all take staff hours.
- **Workers' comp and claims:** home care is a lifting-and-transferring job — comp premiums and the occasional claim are a standing line item once you hit three employees.

PART 10

Getting Your First Clients

In short: Clients come from discharge planners, home-health/hospice partners, the Area Agencies on Aging, CAP/DA case managers, care managers, and senior communities. Build these before you open.

10.1 The referral sources

- **Hospital discharge planners & case managers:** the highest-volume source for post-hospital home support — make same-day starts easy.
- **Skilled home-health & hospice partners:** they refer the non-skilled personal-care hours they don't cover; build reciprocal partnerships.
- **Area Agencies on Aging & the Division of Aging:** NC's 16 AAAs (in the regional Councils of Governments) and the Aging & Disability Resource Connections field families needing in-home help — get listed and known.
- **CAP/DA case-management entities:** for waiver clients, these coordinate services and refer to enrolled providers.
- **Geriatric care managers & senior living communities:** Aging Life Care professionals and assisted-/independent-living communities refer supplemental one-on-one care.
- **NC 2-1-1:** list your services with NC 2-1-1 (dial 211) — a free statewide referral channel.

10.2 What to ask each referral source

"How do you decide which home-care agency to refer a family to?"

"What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?"

"Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?"

"How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?"

"What makes an agency easy to work with, and what gets one dropped from your list?"

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the director & staff

Who	Contact	What to verify / ask
DHSR — Acute & Home Care Licensure	919-855-4620 · DHSR.homecare@dhhs.nc.gov	The pre-application training; the application & \$560 fee; the initial survey
NC DPS — criminal background checks	ncdps.gov	The state/national check within 5 business days of a job offer
Health Care Personnel Registry (HCPR)	info.ncdhhs.gov/dhsr/hcpr	Checking the registry and your reporting policy
Home-care provider training	DHSR-recognized course	The pre-application training requirement
City/County business licensing	your jurisdiction	Any local business license/permit

Business, insurance & billing IDs

Who	Contact	What to verify / ask
NC Secretary of State	919-814-5400 · sosnc.gov	Forming your LLC (\$125); the annual report
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPES / CMS	nppes.cms.hhs.gov	Your Type 2 NPI and the right taxonomy
Insurance agent (home-care program)	trustedchoice.com ; carriers in Part 3.4	Liability + abuse + non-owned auto + dishonesty bond + workers' comp

Funding, EVV & referrals

Who	Contact	What to verify / ask
NCTracks — provider enrollment	800-688-6694 · nctracks.nc.gov	Enrolling your NPI; billing PCS (99509); timely filing
NC Medicaid PCS / NC LIFTSS	833-522-5429 · ncliftss.acentra.com	Policy 3L requirements; assessments; the current per-unit rate
NC Medicaid — EVV / Sandata	medicaid.ncdhhs.gov/EVV	EVV onboarding and which vendor applies to your payer
Medicaid Standard Plans	AmeriHealth Caritas, Carolina Complete, Healthy Blue, UnitedHealthcare, WellCare	Contracting; whether PCS is plan- or Direct-administered for your members
VA Community Care / Optum (Region 1)	844-839-6120 · va.gov/communitycare	The VA Homemaker/HHA program; joining the CCN

Referral & senior-services partners

Who	Contact	Role
Area Agencies on Aging (16 in NC)	NC Division of Aging (ncdhhs.gov)	Local information & referral for older adults
Aging & Disability Resource Connections	via the AAAs	Long-term-care options counseling
NC 2-1-1	dial 211 · nc211.org	Free statewide referral line — list your services
Accreditor (optional) — ACHC/CHAP/TJC	achc.org (Cary, NC) · chapinc.org · jointcommission.org	Whether/when to accredit; payer requirements

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

You can do this

Opening a North Carolina home care agency is very doable — no Certificate of Need for in-home aide services, the moratorium lifted, and a real Medicaid market alongside private pay. Take it one rung at a time: do the provider training, form the business, line up your RN and agency director, get the DHSR license, recruit and screen great aides, launch on private pay, add Medicaid through NCTracks, and build referral relationships. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in NC G.S. Chapter 131E Article 6, 10A NCAC 13J, and current NC Medicaid information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with DHSR, NC Medicaid/NCTracks, and qualified counsel before you rely on it. Confirm the current PCS per-unit rate (an 8% cut took effect Oct 1, 2025) and your EVV setup before you rely on them.